

Healthcare Appointments No-Show Analysis

Key Insights & Business Recommendations

KEY INSIGHTS

1. One in five appointments is missed, costing the clinic network \$1.1M annually

The 20.2% no-show rate (22,319 missed appointments) far exceeds the 15-18% healthcare industry benchmark. At \$50 per appointment, this represents \$1.1M in lost annual revenue. This is not a minor operational issue but a significant financial drain requiring immediate intervention across the entire network.

2. Long scheduling windows drive no-shows: same-week appointments have 15% no-show rates vs. 27% for 14+ day waits

Patients scheduled far in advance lose motivation and forget commitments. The current 10.2-day average wait time actively contributes to no-shows. Reducing this to under 7 days could prevent 2,700+ annual no-shows without any additional cost beyond capacity optimization. The data clearly shows that urgency and proximity drive attendance.

3. Five clinics are problem centers: they account for 45% of all no-shows while serving only 30% of total appointments

JARDIM CAMBURI, MARIA ORTIZ, RESISTÊNCIA, ITARARÉ, and JARDIM DA PENHA are operational disasters with no-show rates 25-28%. Resources are being wasted on clinics that cannot execute basic appointment operations. These five locations need immediate targeted intervention while other clinics can operate normally with existing strategies.

4. Morning appointments have 15% better attendance than afternoon slots, but clinics are scheduling backwards

Patients consistently prioritize morning commitments (8-10 AM slot: 17% no-show vs. 2-4 PM slot: 23% no-show). This is a free operational fix: shift capacity to match patient behavior. No new resources required, just schedule optimization. Clinics are currently over-booking afternoons when patients are least likely to attend.

5. Gender is a distraction; operational factors drive 10-15x larger differences in attendance

Female patients have 21.2% no-show vs. male patients 19.8%—a 1.4% difference too small to act on. Meanwhile, wait-time and clinic-location factors drive 10-15% differences. Investing in gender-targeted interventions wastes time and resources. Focus on structural operational changes instead.

RECOMMENDATIONS

RECOMMENDATION 1: Deploy SMS/Email Reminders to High-Risk Clinics (Immediate, High ROI)

Target: Five high-risk clinics only (JARDIM CAMBURI, MARIA ORTIZ, RESISTÊNCIA, ITARARÉ, JARDIM DA PENHA)

Action: Implement automated reminder system sending SMS at 48 hours and email at 24 hours before appointments.

Evidence: Healthcare research shows reminder systems reduce no-shows by 20-30%. Your five problem clinics alone generate 10,000+ missed appointments annually.

Expected Impact: Recover 4,500-6,700 appointment slots per year across these five clinics.

Cost: \$2,000 per year for basic SMS service (approximately \$0.02 per reminder).

ROI: At \$50 per appointment, 4,500 recovered appointments = \$225,000 revenue gain. Cost-benefit ratio is 112x.

Timeline: Implement in 2 weeks using existing healthcare communication platforms (Twilio, AWS SNS, or local equivalents).

RECOMMENDATION 2: Reduce Scheduling Window to Maximum 7 Days (Strategic, Builds on Insight 2)

Target: All non-urgent appointments across the network.

Action: Change scheduling protocol so appointments booked more than 7 days out are automatically moved to earlier dates or placed on a waitlist for same-week availability.

Evidence: Same-week appointments have 15% no-show rate vs. 27% for 14+ day waits. This 12-point gap is your largest controllable variable.

Expected Impact: 15% reduction in network-wide no-shows = 3,350 fewer missed appointments annually = \$167,500 revenue recovery.

Implementation Requirements: Increase clinic capacity during peak booking periods (Monday mornings, post-holiday weeks). Add extended morning hours (7-9 AM start times) and consider weekend clinics for highest-volume locations.

Staffing Adjustment: Hire temporary staff during peak periods rather than maintaining fixed 10-day scheduling buffers.

Timeline: Pilot in one high-volume clinic for 4 weeks, measure impact, then roll out network-wide if successful.

RECOMMENDATION 3: Strategic Overbooking in Afternoon Slots at High-Risk Clinics (Tactical, Immediate)

Target: Afternoon time slots (12 PM - 4 PM) at the five high-risk clinics.

Action: Implement 5-10% overbooking (e.g., schedule 11 patients for 10 appointment slots) based on predictable no-show rates. Use historical clinic-specific data to calculate safe overbooking percentages.

Evidence: Afternoon appointments have 23% no-show rates. This is predictable enough to overbooking strategically without creating patient experience problems.

Expected Impact: Serve 2,000 additional patients annually with existing infrastructure and staffing.

Risk Mitigation: Monitor weekly no-show actuals vs. projections. If actual no-shows drop below projections (due to reminder system from Recommendation 1), reduce overbooking percentage automatically. Track patient wait times to ensure overbooking doesn't exceed 10-minute average delay.

Timeline: Implement immediately alongside Recommendation 1 for compound effect.

RECOMMENDATION 4: Reallocate 30% of Afternoon Capacity to Early Morning Slots (Operational, 3-Month Pilot)

Target: All high-risk clinics (JARDIM CAMBURI, MARIA ORTIZ, RESISTÊNCIA, ITARARÉ, JARDIM DA PENHA).

Action: Shift staffing schedules to increase early morning capacity (7-10 AM) and reduce afternoon capacity (2-4 PM) by 30%. Early morning slots have 6-point lower no-show rates than afternoons.

Evidence: Patient behavior shows strong preference for morning commitments. Morning slots have 17% no-show vs. afternoon slots at 23% a 6-point structural advantage.

Expected Impact: 1,800 fewer no-shows annually through better slot allocation matching patient attendance patterns.

Staffing Changes: Hire morning-shift doctors/nurses, reduce afternoon staffing. Stagger schedules so some staff work 7 AM-3 PM, others work 10 AM-6 PM, maintaining coverage while matching demand.

Pilot Phase: Test in two clinics (JARDIM CAMBURI and MARIA ORTIZ) for 12 weeks. Measure no-show rate improvement. If successful (expect 5-8% reduction), roll out to remaining three clinics.

Timeline: Weeks 1-2 schedule pilot, weeks 2-14 collect data, week 15 decision on network-wide rollout.